[redacted]

Specimen ID: [redacted]

DOB: **06/08/1949**
Age: **76**
Sex: **Male**

Patient Report

Account Number: **09013070**
Ordering Physician: **R DEMARTINO**

labcorp

Date Collected: **11/19/2025** Date Received: **11/19/2025** Date Reported: **11/29/2025** Fasting: **Yes**

Ordered Items: **CMP14+LP+TP+TSH+5AC+CBC/D/P...; Zn+Cerulo+Cu; Cardiovascular Report; Litholink CKD Program; SARS-CoV-2 Semi-Quant Spike Ab; Insulin; Venipuncture**

Date Collected: **11/19/2025**

CMP14+LP+TP+TSH+5AC+CBC/D/P...

	Test	Current Result and Flag		Previous Result and Date	Units	Reference Interval
	Chemistries [01]					
	Glucose [01]	91			mg/dL	70-99
	Hemoglobin A1c [01]	5.5			%	4.8-5.6
	Please Note: [01]	Prediabetes: 5.7 - 6.4 Diabetes: >6.4 Glycemic control for adults with diabetes: <7.0				
	Uric Acid [01]	4.8			mg/dL	3.8-8.4
				Therapeutic target for gout patients: <6.0		
	BUN [01]	20			mg/dL	8-27
▲	**Creatinine [01]**	**1.32**	**High**		mg/dL	0.76-1.27
▼	**eGFR**	**56**	**Low**		mL/min/1.73	>59
	BUN/Creatinine Ratio	15				10-24
	Sodium [01]	138			mmol/L	134-144
	Potassium [01]	4.6			mmol/L	3.5-5.2
	Chloride [01]	99			mmol/L	96-106
	Carbon Dioxide, Total [01]	24			mmol/L	20-29
	Calcium [01]	9.9			mg/dL	8.6-10.2
	Phosphorus [01]	3.1			mg/dL	2.8-4.1
	Magnesium [01]	2.2			mg/dL	1.6-2.3
	Protein, Total [01]	7.0			g/dL	6.0-8.5
	Albumin [01]	4.0			g/dL	3.8-4.8
	Globulin, Total	3.0			g/dL	1.5-4.5
	Bilirubin, Total [01]	0.5			mg/dL	0.0-1.2
	Alkaline Phosphatase [01]	76			IU/L	47-123
	LDH [01]	172			IU/L	121-224
	AST (SGOT) [01]	14			IU/L	0-40
	ALT (SGPT) [01]	9			IU/L	0-44
	GGT [01]	16			IU/L	0-65
	Iron Bind.Cap.(TIBC)	262			ug/dL	250-450
	UIBC [01]	193			ug/dL	111-343
	Iron [01]	69			ug/dL	38-169
	Iron Saturation	26			%	15-55
	Ferritin [01]	389			ng/mL	30-400
▲	**Cholesterol, Total [01]**	**286**	**High**		mg/dL	100-199
▲	**Triglycerides [01]**	**175**	**High**		mg/dL	0-149
▼	**HDL Cholesterol [01]**	**38**	**Low**		mg/dL	>39
	VLDL Cholesterol Cal	34			mg/dL	5-40

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Date Collected: **11/19/2025**

CMP14+LP+TP+TSH+5AC+CBC/D/P... (Cont.)

▲ **LDL Chol Calc (NIH)**	**214**	**High**	mg/dL	0-99
LDL Calc Comment:	Consider evaluating for Familial Hypercholesterolemia(FH), if clinically indicated.			
▲ **T. Chol/HDL Ratio**	**7.5**	**High**	ratio	0.0-5.0
Please Note:[01]	T. Chol/HDL Ratio Men Women 1/2 Avg.Risk 3.4 3.3 Avg.Risk 5.0 4.4 2X Avg.Risk 9.6 7.1 3X Avg.Risk 23.4 11.0			
▲ **C-Reactive Protein, Cardiac**[01]	**15.89**	**High**	mg/L	0.00-3.00
	Relative Risk for Future Cardiovascular Event Low <1.00 Average 1.00 - 3.00 High >3.00			
Homocyst(e)ine[01]	18.4		umol/L	0.0-19.2
TSH[01]	0.964		uIU/mL	0.450-4.500
Thyroxine (T4)[01]	7.6		ug/dL	4.5-12.0
T3 Uptake[01]	33		%	24-39
Free Thyroxine Index	2.5			1.2-4.9
Triiodothyronine (T3)[01]	85		ng/dL	71-180
Triiodothyronine (T3), Free[01]	2.5		pg/mL	2.0-4.4
▲ **Reverse T3, Serum**[A, 02]	**31.4**	**High**	ng/dL	9.2-24.1
T4,Free(Direct)[01]	1.50		ng/dL	0.82-1.77
Thyroid Peroxidase (TPO) Ab[01]	16		IU/mL	0-34
Thyroglobulin Antibody[03]	<1.0		IU/mL	0.0-0.9
	Thyroglobulin Antibody measured by Beckman Coulter Methodology It should be noted that the presence of thyroglobulin antibodies may not be pathogenic nor diagnostic, especially at very low levels. The assay manufacturer has found that four percent of individuals without evidence of thyroid disease or autoimmunity will have positive TgAb levels up to 4 IU/mL.			
Vitamin D, 25-Hydroxy[01]	41.9		ng/mL	30.0-100.0
	Vitamin D deficiency has been defined by the Institute of Medicine and an Endocrine Society practice guideline as a level of serum 25-OH vitamin D less than 20 ng/mL (1,2). The Endocrine Society went on to further define vitamin D insufficiency as a level between 21 and 29 ng/mL (2). 1. IOM (Institute of Medicine). 2010. Dietary reference intakes for calcium and D. Washington DC: The National Academies Press. 2. Holick MF, Binkley NC, Bischoff-Ferrari HA, et al. Evaluation, treatment, and prevention of vitamin D deficiency: an Endocrine Society clinical practice guideline. JCEM. 2011 Jul; 96(7):1911-30.			
Fibrinogen Activity[01]	479		mg/dL	193-507
.[01]				
CBC, Platelet Ct, and Diff[01]				

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CMP14+LP+TP+TSH+5AC+CBC/D/P... (Cont.)

	WBC [01]	9.8		x10E3/uL	3.4-10.8
	RBC [01]	5.67		x10E6/uL	4.14-5.80
	Hemoglobin [01]	16.9		g/dL	13.0-17.7
▲	**Hematocrit [01]**	**51.3**	**High**	%	37.5-51.0
	MCV [01]	91		fL	79-97
	MCH [01]	29.8		pg	26.6-33.0
	MCHC [01]	32.9		g/dL	31.5-35.7
	RDW [01]	13.2		%	11.6-15.4
	Platelets [01]	410		x10E3/uL	150-450
	Neutrophils [01]	68		%	Not Estab.
	Lymphs [01]	20		%	Not Estab.
	Monocytes [01]	8		%	Not Estab.
	Eos [01]	2		%	Not Estab.
	Basos [01]	1		%	Not Estab.
	Neutrophils (Absolute) [01]	6.8		x10E3/uL	1.4-7.0
	Lymphs (Absolute) [01]	1.9		x10E3/uL	0.7-3.1
	Monocytes(Absolute) [01]	0.8		x10E3/uL	0.1-0.9
	Eos (Absolute) [01]	0.2		x10E3/uL	0.0-0.4
	Baso (Absolute) [01]	0.1		x10E3/uL	0.0-0.2
	Immature Granulocytes [01]	1		%	Not Estab.
	Immature Grans (Abs) [01]	0.1		x10E3/uL	0.0-0.1
	. [01]				
	Urinalysis Gross Exam [01]				
	Specific Gravity [01]	1.022			1.005-1.030
	pH [01]	5.5			5.0-7.5
	Urine-Color [01]	Yellow			Yellow
	Appearance [01]	Clear			Clear
	WBC Esterase [01]	Negative			Negative
	Protein [01]	Trace			Negative/Trace
	Glucose [01]	Negative			Negative
▶	**Ketones [01]**	**Trace**	**Abnormal**		Negative
	Occult Blood [01]	Negative			Negative
	Bilirubin [01]	Negative			Negative
	Urobilinogen,Semi-Qn [01]	0.2		mg/dL	0.2-1.0
	Nitrite, Urine [01]	Negative			Negative
	Microscopic Examination [01]				

Microscopic not indicated and not performed.

Zn+Cerulo+Cu

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Copper, Serum or Plasma [A, 02]	112		ug/dL Detection Limit = 5	69-132
Zinc, Plasma or Serum [A, 02]	67		ug/dL Detection Limit = 5	44-115

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Date Collected: **11/19/2025**

Zn+Cerulo+Cu (Cont.)

▲ **Ceruloplasmin** [01]	**31.3**	**High**		mg/dL	16.0-31.0

Cardiovascular Report

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Interpretation [04]	Note Supplemental report is available.			
PDF [04]	Not applicable			

Litholink CKD Program

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Interpretation [04]	Note ---------- CHRONIC KIDNEY DISEASE: EGFR, BLOOD PRESSURE, AND PROTEINURIA ASSESSMENT We presume eGFR has been less than 60 mL/min/1.73mE2 on at least two occasions spaced at least 3 months apart. Current eGFR is 56 mL/min/1.73mE2 corresponding to CKD stage 3a. Potassium is within goal, 4.6 mmol/L. Hemoglobin A1C is 5.5 %; diabetic status is unknown. Refer to ADA guidelines for clinical practice recommendations. EGFR, BLOOD PRESSURE, AND PROTEINURIA TREATMENT SUGGESTIONS - Guidelines recommend a target blood pressure of 120/80 mmHg or less to reduce cardiovascular risk and CKD progression. Assessment of albuminuria (urine albumin:creatinine ratio or urine protein:creatinine ratio preferred) is recommended at least annually in CKD patients for staging and disease prognosis. EGFR, BLOOD PRESSURE, AND PROTEINURIA FOLLOW-UP - fasting Renal Panel within 12 months; Spot Urine Panel is recommended by guidelines, at least yearly; BONE and MINERAL ASSESSMENT Phosphorus is within goal, 3.1 mg/dL. Calcium is within goal, 9.9 mg/dL. Carbon Dioxide is within goal, 24 mmol/L. Vitamin D is adequate (41.9 ng/mL). BONE and MINERAL FOLLOW-UP - 25-Hydroxy Vitamin D within 12 months; fasting PTH with Renal Panel is recommended by guidelines, at least yearly; LIPIDS ASSESSMENT LDL-C is very high, 214 mg/dL. Triglyceride is borderline high, 175 mg/dL. Non-HDL Cholesterol is very high, 248 mg/dL. HDL-C is low, 38 mg/dL. Markedly elevated LDL-C raises the possibility of familial hypercholesterolemia. A family history of high cholesterol and heart disease in first degree relatives should be obtained. LIPIDS TREATMENT SUGGESTIONS - Therapeutic lifestyle changes are always valuable to maintain optimal blood lipid status (diet, exercise, weight management). Begin statin. If statin already in use, consider increasing dose to achieve at least a 50% LDL			

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Litholink CKD Program (Cont.)

reduction from baseline. High intensity statin is preferred. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant), niacin, and/or fish oil.
LIPIDS FOLLOW-UP
-
fasting Lipid Panel within 3 months;
ANEMIA ASSESSMENT
Hemoglobin is normal, 16.9 g/dL. TSAT is within goal, 26.0 %. Ferritin is within goal, 389 ng/mL. Hemoglobin target assumes ESA is not in use.
ANEMIA TREATMENT SUGGESTIONS
-
No specific change of treatment is indicated at this time.
ANEMIA FOLLOW-UP
-
CBC within 12 months;

DISCLAIMER
These assessments and treatment suggestions are provided as a convenience in support of the physician-patient relationship and are not intended to replace the physician's clinical judgment. They are derived from national guidelines in addition to other evidence and expert opinion. The clinician should consider this information within the context of clinical opinion and the individual patient.
SEE GUIDANCE FOR CHRONIC KIDNEY DISEASE PROGRAM: Kidney Disease Improving Global Outcomes (KDIGO) clinical practice guidelines are at http://kdigo.org/home/guidelines/.
National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI (TM)), with its limitations and disclaimers, are at www.kidney.org/professionals/KDOQI. This program is intended for patients who have been diagnosed with stages 3, 4, or pre-dialysis 5 CKD. It is not intended for children, pregnant patients, or transplant patients.

PDF [04]	.

SARS-CoV-2 Semi-Quant Spike Ab

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
SARS-CoV-2 Semi-Quant Spike Ab [B, 01]	See Dilution		U/mL	Negative<0.8
SARS-CoV-2 Spike Ab Dilution [B, 01]	11050		U/mL	Negative<0.8
SARS-CoV-2 Spike Ab Interp [B, 01]	Positive			

Antibodies against the SARS-CoV-2 spike protein receptor binding domain (RBD) were detected. It is yet undetermined what level of antibody to SARS-CoV-2 spike protein correlates to immunity against developing symptomatic SARS-CoV-2 disease. Studies are underway to measure the quantitative levels of specific SARS-CoV-2 antibodies following vaccination. Such studies will provide valuable insights into the correlation between protection from vaccination and antibody levels.
Roche Elecsys Anti-SARS-CoV-2 S

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Date Collected: **11/19/2025**

Insulin

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Insulin [01]	6.4		uIU/mL	2.6-24.9

Previous Result
The Previous Result is listed for the most recent test performed by Labcorp in the past 5 years where there is sufficient patient demographic data to match the result to the patient. Results from certain tests are excluded from the Previous Result display.

Icon Legend
▲ Out of Reference Range ■ Critical or Alert

Footnotes/Disclaimers
A: This test was developed and its performance characteristics determined by Labcorp. It has not been cleared or approved by the Food and Drug Administration.
B: This test has not been FDA cleared or approved. This test has been authorized by FDA under an Emergency Use Authorization (EUA). This test is only authorized for the duration of the declaration that circumstances exist justifying the authorization of emergency use of in vitro diagnostics for detection and/or diagnosis of COVID-19 under Section 564(b)(1) of the Act, 21 U.S.C. 360bbb-3(b)(1), unless the authorization is terminated or revoked sooner. This test has been authorized only for detecting the presence of antibodies against SARS-CoV-2, not for any other viruses or pathogens.

Performing Labs
01: PDLCA - Labcorp Phoenix, 5005 S 40th Street Ste 1200, Phoenix, AZ 85040-2969 Dir: Earle Collum, MD
02: BN - Labcorp Burlington, 1447 York Court, Burlington, NC 27215-3361 Dir: Sanjai Nagendra, MD
03: SPOWA - Labcorp Spokane, 110 W Cliff Dr. Ste 100-200, Spokane, WA 99204-3614 Dir: Shefali Goyal, MD
04: LITNC - Labcorp Clinical / Digital, 10 Moore Drive, Durham, NC 27709-0009 Dir: Jennifer Ennis, MD
For inquiries, the physician may contact Branch: 800-762-4344 Lab: 800-788-9743

Patient Details
Hiestand, Denis

Phone: **702-789-9706**
Date of Birth: **06/08/1949**
Age: **76**
Sex: **Male**
Patient ID: **438318**
Alternate Patient ID: **438318**

Physician Details
R DEMARTINO
PROFESSIONAL CO-OP SERVICES
2700 N 29th Ave Ste 308, Hollywood, FL, 33020

Phone: **866-999-4041**
Account Number: **09013070**
Physician ID: **1265468359**
NPI: **1265468359**

Specimen Details
Specimen ID: **323-847-1458-0**
Control ID: **00631669**
Alternate Control Number: **00631669**
Date Collected: **11/19/2025 0713 Local**
Date Received: **11/19/2025 0000 ET**
Date Entered: **11/19/2025 1428 ET**
Date Reported: **11/29/2025 2206 ET**

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SEE GUIDANCE FOR CARDIOVASCULAR REPORT: Grundy SM et al. 2018 Multisociety guideline on the management of blood cholesterol: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines. J Am Coll Cardiol 2019; 73: e285-350; Contois et al. Clin Chem 2009; 55(3):407-419; Brunzell et al. Diabetes Care 2008; 31(4):811-82.
SEE GUIDANCE FOR CHRONIC KIDNEY DISEASE PROGRAM: Kidney Disease Improving Global Outcomes (KDIGO) clinical practice guidelines are at http://kdigo.org/home/guidelines/. National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI (TM)), with its limitations and disclaimers, are at www.kidney.org/professionals/KDOQI. This program is intended for patients who have been diagnosed with stages 3, 4, or pre-dialysis 5 CKD. It is not intended for children, pregnant patients, or transplant patients.

Note: *Please refer to your LabCorp Report for all results as well as any test-specific and specimen-specific comments.*

Chronic Kidney Disease

Analysis & Treatment Suggestions

GENDER	DIABETES	CURRENT eGFR	MOST RECENT CKD STAGE
M	?	**56**	**3a**

Prognosis of CKD by GFR And Albuminura Categories

GFR Categories: (ml/min/1.73m²)			A1	A2	A3
G1	>=90	NORMAL OR HIGH			
G2	60-89	MILDLY DECREASED			
G3a	45-59	MILDLY TO MODERATELY DECREASED			
G3b	30-44	MODERATELY TO SEVERELY DECREASED			
G4	15-29	SEVERELY DECREASED			
G5	<15	KIDNEY FAILURE			

Persistent Albuminura Categories

A1 = NORMAL TO MILDLY INCREASED; ACR <30 ug/mg; PCR <=200 mg/g
A2 = MODERATELY INCREASED; ACR 30-300 ug/mg; PCR 201-500 mg/g
A3 = SEVERELY INCREASED; ACR >300 ug/mg; PCR >500 mg/g

= LOW RISK = MODERATELY INCREASED RISK
= HIGH RISK = VERY HIGH RISK

● = PATIENT'S RESULT

Kidney Disease: Improving Global Outcomes (KDIGO) CKD Work Group. KDIGO 2012 Clinical Practice Guideline for the Evaluation and Management of Chronic Kidney Disease. *Kidney inter.; Suppl.* 2013; 3: 1-150.

eGFR, Blood Pressure, and Proteinuria

We presume eGFR has been less than 60 mL/min/1.73mE2 on at least two occasions spaced at least 3 months apart. Current eGFR is 56 mL/min/1.73mE2 corresponding to CKD stage 3a. Potassium is within goal, 4.6 mmol/L. Hemoglobin A1C is 5.5 %; diabetic status is unknown. Refer to ADA guidelines for clinical practice recommendations.

Treatment Suggestions

Guidelines recommend a target blood pressure of 120/80 mmHg or less to reduce cardiovascular risk and CKD progression. Assessment of albuminuria (urine albumin:creatinine ratio or urine protein:creatinine ratio preferred) is recommended at least annually in CKD patients for staging and disease prognosis.

Bone & Mineral

Phosphorus is within goal, 3.1 mg/dL. Calcium is within goal, 9.9 mg/dL. Carbon Dioxide is within goal, 24 mmol/L. Vitamin D is adequate (41.9 ng/mL).

Lipids

LDL-C is very high, 214 mg/dL. Triglyceride is borderline high, 175 mg/dL. Non-HDL Cholesterol is very high, 248 mg/dL. HDL-C is low, 38 mg/dL. Markedly elevated LDL-C raises the possibility of familial hypercholesterolemia. A family history of high cholesterol and heart disease in first degree relatives should be obtained.

Treatment Suggestions

Therapeutic lifestyle changes are always valuable to maintain optimal blood lipid status (diet, exercise, weight management). Begin statin. If statin already in use, consider increasing dose to achieve at least a 50% LDL reduction from baseline. High intensity statin is preferred. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant), niacin, and/or fish oil.

Anemia

Hemoglobin is normal, 16.9 g/dL. TSAT is within goal, 26.0 %. Ferritin is within goal, 389 ng/mL. Hemoglobin target assumes ESA is not in use.

Treatment Suggestions

No specific change of treatment is indicated at this time.

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Follow-Up Suggestions for CKD

Recommended by guidelines, at least yearly

- Spot Urine Panel
- Fasting PTH with Renal Panel

3 months

- Fasting Lipid Panel

12 months

- Fasting Renal Panel
- 25-Hydroxy Vitamin D
- CBC

Cardiovascular Report

Patient Assessment

Current available clinical information suggests the patient's risk is at least HIGH. Your patient has an eGFR result (<60) that could indicate the presence of a CHD risk equivalent (chronic kidney disease). Markedly elevated LDL-C raises the possibility of familial hypercholesterolemia. Two additional major risk factors are present (age over 45 and HDL-C less than 40).

High sensitivity CRP and Lp(a) results identify sources of residual risk that may warrant more intensive therapy. Hs-CRP (15.89 mg/L) is markedly elevated; consider non-cardiac causes.

Insulin resistance, obesity, excessive alcohol use, smoking, nephrotic syndrome, liver disease, and certain medications can cause secondary dyslipidemia. Consider evaluation if clinically indicated.

Therapeutic lifestyle changes are always valuable to achieve optimal blood lipid status (diet, exercise, weight management).

Lipid Management

Select one patient risk category based upon medical history and clinical judgment. Additional risk factors such as personal or family history of premature CHD, smoking, and hypertension modify a patient's goals of therapy. In CVD prevention, the intensity of therapy should be adjusted to the level of patient risk. MODERATE intensity statin therapy generally results in an average LDL-C reduction of 30% to less than 50% from the untreated baseline. Examples include (daily doses): atorvastatin 10-20 mg, rosuvastatin 5-10 mg, simvastatin 20-40 mg, pravastatin 40-80 mg, lovastatin 40 mg. HIGH intensity statin therapy generally results in an average LDL-C reduction of 50% or more from the untreated baseline. Examples include (daily doses): atorvastatin 40-80 mg and rosuvastatin 20 mg.

▽ = PATIENT'S RESULT

ANALYTE / RESULT	**Patient Risk Category** (select one)		
	LOW	**INTERMEDIATE**	**HIGH**
LDL-C **214 mg/dL**	▽ 160	▽ 100 130	▽ 70 100
non-HDL **248 mg/dL**	▽ 190	▽ 130 160	▽ 100 130
Lipid Assessment	LDL-C is very high, 214 mg/dL. Non-HDL Cholesterol is very high, 248 mg/dL.	LDL-C is very high, 214 mg/dL. Non-HDL Cholesterol is very high, 248 mg/dL.	LDL-C is very high, 214 mg/dL. Non-HDL Cholesterol is very high, 248 mg/dL.
Treatment Suggestions	Please refer to assessment and treatment suggestions under high risk category.	Please refer to assessment and treatment suggestions under high risk category.	Begin statin. If statin already in use, consider increasing dose to achieve at least a 50% LDL reduction from baseline. High intensity statin is preferred. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant), niacin, and/or fish oil.

DISCLAIMER: *These assessments and treatment suggestions are provided as a convenience and are neither comprehensive nor intended to replace the physician's clinical judgment. They do not include information such as family history, personal history, or physical findings as would be obtained by the clinician during patient evaluation because LabCorp does not have access to the complete patient medical record.*

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PATIENT	DATE OF BIRTH	GENDER	CKD STAGE	DATE OF SERVICE	PHYSICIAN
HIESTAND, DENIS	**06/08/1949**	**M**	**3a**	**11/19/2025**	LabCorp Account #: 09013070

Your Test Results

WITHIN GOAL | BORDERLINE | OUT OF GOAL

Test	Your Results	Comments
Blood Lipids		
LDL CHOLESTEROL is the bad cholesterol that can clog your arteries.	**214** Ref. Range: 0 to 99 mg/dL	**LOWERING LDL IS IMPORTANT** to protect your heart and blood vessels. Your doctor may prescribe or increase medicine to lower your cholesterol. Remember to take all medicines as prescribed. Weight loss, exercise (at least 30 minutes 5 times a week), eating less trans and saturated fats, and quitting smoking can also help to lower cholesterol.
Kidney Health		
eGFR estimates how well your kidneys are filtering blood. The higher the number, the better your kidneys are working.	**56** Ref Range: >59 mL/min/1.73mE2	**YOUR RESULT COULD MEAN YOU HAVE STAGE 3a CHRONIC KIDNEY DISEASE (CKD).** There are 5 stages of CKD. See below for more information.
POTASSIUM helps keep your heart and muscles working properly. High or low levels can be dangerous.	**4.6** Ref. Range: 3.5 to 5.2 mmol/L	**YOUR POTASSIUM IS NORMAL.** To keep it in the normal range, make sure you avoid foods high in potassium. For more information, visit www.kidney.org/atoz/content/potassium.cfm.
PHOSPHORUS works with calcium to build strong, healthy bones and keep the rest of the body healthy. Too much phosphorus can damage the heart and blood vessels.	**3.1** Ref. Range: 2.8 to 4.1 mg/dL	**YOUR PHOSPHORUS IS NORMAL.** To keep it in the normal range, continue to avoid foods high in phosphorus. For more information, visit www.kidney.org/atoz/content/phosphorus.cfm. Also remember to take your medicines as prescribed by your doctor.
HEMOGLOBIN is a test to look for anemia, or low blood count. Low hemoglobin levels may cause you to feel tired or have low energy.	**16.9** Male Range: 13 to 17 g/dL	**YOUR HEMOGLOBIN IS NORMAL.** Because low hemoglobin is common and can happen over time, it is important to follow up regularly with your doctor for continued monitoring.

YOUR eGFR IS 56, YOU ARE IN STAGE 3a CKD. THINGS YOU NEED TO KNOW:

- CKD means your kidneys show decreased function which may be caused by aging, diabetes, high blood pressure, or many other diseases. CKD can lead to heart disease.

DISCLAIMER: You should discuss this information with your physician. Labcorp does not have a doctor-patient relationship with you, nor does it have access to a complete medical history or physical examination conducted by a physician that would be necessary for a complete diagnosis and comprehensive treatment plan. Neither you nor your physician should rely solely on this guidance. Bolded result descriptions in "Comments" consider either the reference range or target range for the test result. Reference range refers to the Labcorp reference interval. Target range refers to the guideline-suggested goal. REFERENCES:National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI) at www.Kidney.org and Kidney Disease Improving Global Outcomes (KDIGO) at http://kdigo.org. Adapted from: https://www.niddk.nih.gov/-/media/Files/Health-Information/Health-Professionals/Kidney-Disease/Your_Kidney_Test_Results_EN.pdf.

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- Even if you feel fine, you may be at risk for problems such as high blood pressure, high cholesterol, anemia, and bone disease. Your doctor should test you for these problems.
- Avoid taking NSAIDs (e.g. ibuprofen, Advil®, Motrin®, Aleve®) as these may worsen your kidney function.
- To learn more, visit the National Kidney Foundation's website: www.kidney.org/kidneydisease.

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